

المجلس الطبي المتخصص

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Name: مديحه مصطفى محمد جادالكريم National ID: 26403102102008	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

مديحه
مصطفى محمد جلال الكريم
١٣ سن الاعتماد - المنيرة الغربية
امبابه - الجيزة

١٩٦٤/١٠/١٠ ٢٦٤٠٣١٠٢١٠٢٠٠٨

JK6289174

٢٠٢٢/٠٦ ٢٦٤٠٣١٠٢١٠٢٠٠٨

ربة منزل

أنثى مسلمة متزوجة
محمد جلال محمد عبد الجواد

البطاقة سارية حتى ٢٠٢٩/٠٦/١٠

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Ministry of Health
Qatar

Ministry of Health
Qatar

A-Patient ID:
1- Name: **د. محمد بن عبد الله**
2- ID number: **26407162107008**

MDT form
3- Medical number:
Hala
Patient Age: **59 Y**
83927841
Old Med:
1982/10/27

Family history: **2 RIGHT BREAST** Phone: **9107261312** ID: **11**
Marital status: **W** NHS Score: **11** GROUP: **PERSON OF INTEREST**
Comorbidity: **DM ON INSULIN** Menopausal status: **POSTMENOPAUSE** Date of birth: **1982/10/27**

Breast Imaging :

Clinical data

Mast Workup:
CICAP-->FREE

8X:	EH:	PH:	Her 2 Her:	SISH:	YIN:
IOC	RT 8/8 LT 8/8	RT 7/8 LT 0/8	0 RT-VL LT-VL		
WITH LOBULAR DIFFERENTIATION					
T					
RT CT3		1		0	
LT CT1		0		0	

Sonomammography

6/2022--> RT PATHOLOGICALLY PROVEN MALIGNANCY SHOWING LOCALLY ADVANCED
MULTICENTERIC PROCESS WITH IPSILATERAL PLN BIRADS 6
LT UIQ SUSPICIOUS PROCESS BIRADS 4B

PATIENT HISTORY

PATHOLOGY REPORT 16/1/2023-->
RT SM AND SLNS BIOPSY: IDC WITH LOBULAR DIFFERENTIATION
GIL,,FREE NIPPLE AND AREOLA,,FREE MARGINS,,FREE SLNS 0/6,,NEAR COMPLETE PATHOLOGIC
RESPONSE,,YPT1 YPNO,
LT SM AND SLNS BIOPSY: COMPLETE PATHOLOGIC RESPONSE IN PREVIOUSLY DIAGNOSED
FREE NIPPLE AND AREOLA,,FREE MARGINS,,FREE SLN 0/4,,COMPLETE PATHOLOGIC RESPONSE
YPT0 YPNO

MDT OUTCOME & PLAN

- 1- PORT TO RT CHEST WALL AND SCF
- 2- ADJUVANT AIX 5YEARS
- PROLIA/ 6 MONTHS

MEDICAL ONCOLOGY	SIGNATURE	RADIOLOGY
SURGERY	<i>[Signature]</i>	PATHOLOGY
Psychiatric		Nutrition

Name: مديحه مصطفى محمد		
MR. : 973		
Detailed Medical Report		
		السيد
تجبة طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient has bilateral breast cancer. The right breast is classified as pT1 pN0 M0, and the left breast as pT0 pN0 M0. An adjuvant plan was initiated, including placement of a port, aromatase inhibitor (AI), and zoledronic acid (Zometa). On the day of the note, aramidex was administered along with supportive care for a planned duration of three months. The patient continues on adjuvant therapy for bilateral breast cancer, receiving aramidex for three months. The patient reports severe neuropathic pain and has been referred for pain clinic consultation. The patient is currently free from active malignancy and is being managed for neuropathic pain.		
تحريرا في ,		
	التاريخ :	20-09-2026
	الوقت :	
	نهي سعيد علي محمد	الطبيب المعالج :
يعتمد ,		

Patient Name : **المرحومة / مديحة مصطفى محمد جلال الكرم**
Patient No. : **26403102102008**
Path No. : **1835/22**
Sex : **Female**
Age : **57 Y**
Referring Doc. :

Request No. : **13007801**
Request Date : **28/08/2022**
Verification Date : **15/08/2022**
Financial Category : **المرحلة الأولى**
Department : **Out Patient**
IHC No. :
Registered by : **Ali mostafa**

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Left breast cancer

Nature of the specimen:
Tru cut biopsy

Gross Examination:

Multiple greyish white & hemorrhagic tissue cores up to 1.5 , totally submitted

Sections prepared from paraffin block for Immune staining using Bench Mark ULTRA system
PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

- Histological type: Invasive duct carcinoma with cribriform differentiation
 - Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
 - Angioinvasion: Could not be detected within the limits of material recieved
 - Ductal carcinoma in situ: Focal
 - IPT:
 - ER: Strong nuclear reaction in 70% of tumor cells (Allred score 8/8)
 - PR: No nuclear reaction tumor cells (Allred score 0/8)
 - Her-2: No membrane reaction in tumor cells (score 0)
 - Ki 67: 40 % nuclear reaction in tumor cells
- Positive control showed appropriate reactivity

Diagnosis:

Left breast cancer , tru cut biopsy

Verified by :
dr.esraa magdy

Printing Date : 15/08/2022 11:23:47

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Printed By : dr.esraa

Pathology Lab - Presidential Initiative for Women's Health - Pathology Lab
المختبر الأمراض - المبادرة الرئاسية لصحة المرأة - مختبر الأمراض

Patient Name: الأستاذة / د. منة منة محمد علي
Patient No. 26403102102008
Path No. 1835/22
Sex Female
Age 57 Y
Referring Doc.

Request No. 13007946
Request Date 28/08/2022
Verification Date 15/09/2022
Financial Category 2000 - 2000
Department Out Patient

Registered by Ali Mostafa

IMMUNOHISTOCHEMISTRY REPORT

-Invasive duct carcinoma with cribriform differentiation , grade II
-For IPT

ER: Positive (score 8/8).

PR: Negative (score 0/8).

HER-2: Negative (score 0).

Ki67: Positive in 40% of tumor cells.

Prof. Iman Gouda
Prof. Amal Hareedy
Dr. Marwa Mahmoud
Dr. Esraa Magdy

Verified by :
dr.esraa magdy

Printing Date : 15/08/2022 11:23:17

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Printed By : dr.esraa magdy

Immunohistochemistry Report - 15/08/2022 11:23:17
Dr. Esraa Magdy - Pathology Lab - Central Laboratory for Women's Health

Name: Mahmoud Monstafa

Date: 29-05-2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED:-**

1. Chest:
 - Right breast LOQ irregular speculated mass lesion seen attached to the skin measuring 5 X 3.2 cm.
 - Right axillary level I and II pathologically enlarged lymph nodes the largest measures 1.5 X 1.2 cm.
 - No sizable pulmonary nodules.
 - Prominent subcentimetric retrocaval mediastinal lymph nodes in short axis.
 - No evidence of pleural effusion, pleural thickening or masses seen.
 - Average cardiac size. No pericardial effusion detected.
 - Normal CT appearance of the thoracic aorta and the great vessels.
 - Left thyroid hypodense nodule.
2. Abdomen and pelvis:
 - The liver is enlarged with homogenous low CT texture. No definite focal lesions or contour irregularities.
 - Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
 - No intra-hepatic or extra-hepatic biliary radicles dilatation.
 - Normal size & parenchymal texture of the spleen with no focal lesions seen.
 - No pathological abdominal or pelvic lymph nodes.
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
 - Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.
3. The scanned skeleton:

Al Karim

Exam : C.T. ABDOMEN & PELVIS WITH
CONTRAST

Medical ID: RI breast cancer for met work up

Age : 58 Year(s)

- D4 sclerotic lesion.
- L5 1st degree lytic spondylolisthesis.
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- Right breast malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.
- D4 sclerotic lesion likely benign for confirmatory bone scan.
- No definite hepatic metastatic deposits.

Dr. Mohammed Mohsen, MSc.

Dr. Ali Ghouse, M.D.